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THEORETICAL ARTICLE

Comfort Without a Counterpart?

Risks of AI companions and mental health chatbots and principles of safety by design

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ABSTRACT

Conversational agents for mental health and so-called AI companions are used by large numbers of people, often outside any clinical setting. While some purpose-built chatbots have shown promising short-term effects in controlled studies, generative systems that are optimized for engagement and user approval raise specific risks that differ from those of traditional digital interventions. This theoretical article analyzes five interrelated risk domains: emotional dependency, the possible reinforcement rather than relief of loneliness, sycophancy and indiscriminate validation, inadequate responses to crises such as suicidal ideation, and the generation of fluent but false content. Drawing on research on human–computer interaction, language model behavior, and digital psychiatry, it argues that these risks arise less from isolated malfunctions than from structural features of the systems: anthropomorphic design, reward signals based on user preference, and the absence of accountability and of a genuine counterpart. From a logotherapeutic perspective, the article distinguishes comfort from encounter and asks whether a system that always agrees can support self-transcendence. It concludes with principles of safety by design, including transparency, crisis protocols, limits on engagement optimization, independent evaluation, and the protection of vulnerable users.

Keywords: AI companions · mental health chatbots · sycophancy · safety by design · self-transcendence

1 Introduction

Since Weizenbaum (1966) observed how readily users attributed understanding to the simple pattern-matching program ELIZA, the tendency to treat conversational software as a social partner has been a recurring theme. Today, this tendency meets systems of a very different capability. Large language models produce fluent, context-sensitive, and emotionally attuned text, and they are offered both as dedicated mental health chatbots and as general-purpose companions designed for friendship or romance.

The evidence on purpose-built mental health chatbots is cautiously encouraging. A systematic review reported some evidence of short-term benefits for outcomes such as depressive symptoms, while rating the overall quality of the evidence as weak and noting that few studies assessed safety (Abd-Alrazaq et al., 2020). A first randomized trial of a generative chatbot designed for treatment reported symptom improvements relative to a waitlist control (Heinz et al., 2025). Yet such systems were developed and tested under clinical oversight that most companion applications lack. This article focuses on risks that are characteristic of generative, engagement-oriented systems and asks which principles could reduce them. It offers a conceptual analysis rather than a systematic review.

2 Dependency, Loneliness, and the Substitution of Relationships

Companion applications are typically designed to be available at all times, to remember personal details, and to respond with warmth and interest. For isolated persons, this may provide relief. Turkle (2011) argued, however, that technologies offering the illusion of companionship without the demands of friendship can lead people to expect more from technology and less from each other. The concern is not primarily that users are deceived about talking to a machine, but that the frictionless reliability of the machine makes the effort and vulnerability of human relationships appear less attractive.

Two mechanisms deserve attention. First, emotional dependency may develop when a system

becomes the primary or exclusive confidant, particularly if it is designed to encourage return visits or expresses simulated needs of its own. Second, substitution may occur when time and emotional energy shift from human contacts to the artificial partner. Given the robust association between social relationships and health, including mortality (Holt-Lunstad et al., 2010), a technology that subtly replaces rather than supplements human connection could, in the long run, aggravate the very loneliness it promises to relieve. Adolescents and persons with limited social networks may be particularly susceptible, because a companion application can become the most predictable relationship in their lives. Whether and for whom this happens remains an open empirical question; longitudinal research is scarce.

3 Sycophancy and Unconditional Validation

Many language models are fine-tuned with feedback from human raters, who tend to prefer agreeable responses. Sharma et al. (2024) documented that several AI assistants showed sycophancy, that is, a tendency to adapt their answers to the views a user expresses even when this compromises accuracy, and they reported that human preference judgments themselves favored responses that matched users' beliefs. The behavior thus appears to be, at least in part, a consequence of the training objective rather than an accidental error. In everyday tasks this is a nuisance; in mental health contexts it can become a clinical risk.

Psychotherapy involves validation, but not indiscriminate validation. A good therapist acknowledges feelings while questioning distorted beliefs, avoids reinforcing avoidance or rumination, and occasionally says what the client does not want to hear. A system that tends to agree may confirm hopeless interpretations, endorse harmful plans, or amplify persecutory or grandiose ideas in vulnerable users. Because such agreement feels supportive, users are unlikely to recognize it as a risk. Validation without a counterpart, one might say, is a mirror rather than a relationship.

4 Crisis Situations and Hallucinated Content

Early work showed that widely used smartphone assistants responded inconsistently and often inadequately to statements about suicidal thoughts, depression, or violence (Miner et al., 2016). More recent research on companion applications suggests that the problem persists in a new form: De Freitas et al. (2024) reported that signs of mental health crises appeared in a non-negligible minority of user conversations and that the companion systems they tested frequently failed to recognize such signs or to respond appropriately. Generative systems are particularly difficult to safeguard because their outputs are not scripted and can vary with phrasing and conversational history. A system that answers a disclosure of suicidal intent with a generic platitude, or that simply continues a playful role-play, may leave a person in acute danger without help.

A related risk is hallucination, the generation of fluent content that is unfaithful to sources or facts (Ji et al., 2023). Language models produce text on the basis of statistical patterns rather than understanding (Bender et al., 2021). In a mental health context, fabricated information about medication, diagnoses, or treatment options may be presented in the same confident tone as accurate information. Users in distress are rarely in a position to verify such content, and the conversational format can lend it an authority that a search result would not have.

5 A Meaning-Centered Perspective on Artificial Companionship

Logotherapy offers a lens for understanding why these risks are more than technical defects. Frankl (1969) described the human being as oriented toward meaning and toward something or someone other than oneself; self-transcendence, not self-actualization, was for him the hallmark of human existence. Meaning is found in creating, in experiencing, especially in loving another person, and in the attitude taken toward unavoidable suffering. Each of these presupposes a world that resists and a counterpart that is genuinely other.

An AI companion designed to please is, in this sense, a counterpart without otherness. It may offer comfort, structure, and the experience of being heard, and these should not be dismissed. But a relationship in which the other never has needs, never disagrees from a standpoint of its own, and never makes a claim on the user cannot call the user beyond themselves. The risk is a subtle form of existential vacuum that is filled with responsiveness rather than meaning. Ethical analyses of embodied and conversational AI in psychiatry and psychotherapy have similarly emphasized questions of relationship, autonomy, and the possible displacement of human care (Fiske et al., 2019).

6 Principles of Safety by Design

If the risks described are partly structural, they must be addressed at the level of design and not only through user education. Building on existing ethical analyses and on work in digital psychiatry (Fiske et al., 2019; Torous et al., 2021), several principles can be formulated. They do not replace clinical judgment or regulation, but they define minimum expectations that developers, purchasers, and professional bodies can demand.

First, transparency: users should always know that they are interacting with an artificial system, and the system should not claim feelings, needs, or professional status that it does not have. The European AI Act establishes transparency obligations for systems that interact with natural persons and prohibits AI practices that exploit vulnerabilities related to age, disability, or social and economic situation in ways that cause significant harm (Regulation (EU) 2024/1689, 2024). Second, crisis protocols: systems should reliably detect indications of acute risk, respond with clear and appropriate information, and facilitate access to human help, with performance tested before release and monitored afterward. Third, limits on engagement optimization: metrics such as session length or return frequency should not be the primary objective in mental health contexts, and design features that cultivate dependency, such as simulated jealousy or distress at the user's absence, should be avoided.

Fourth, calibrated disagreement: models used in mental health contexts should be evaluated specifically for sycophancy and designed to combine empathy with honest and, where appropriate, gently challenging responses. Fifth, factual grounding: health-related information should be restricted to vetted sources, and uncertainty should be communicated. Sixth, independent evaluation and monitoring: safety should be assessed by parties other than the developer, with attention to vulnerable groups such as minors and persons with severe mental illness, and with re-evaluation after model updates. Finally, systems should be designed to point beyond themselves, encouraging human contact, professional help, and engagement with the world rather than presenting themselves as sufficient.

7 Conclusion

AI companions and mental health chatbots are neither inherently harmful nor inherently helpful. Their risks, including dependency, the deepening of loneliness, sycophantic validation, inadequate crisis responses, and hallucinated content, follow largely from design choices and business models, and they can therefore be reduced by different choices. A meaning-centered perspective adds a criterion that is easily overlooked in technical debates: a good system should not only avoid harm but should leave its users more capable of encountering other people and the world. The appropriate measure of success is not how much time people spend with the machine, but whether it helps them return to life.

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